intersex
NIGERIA
CENTER FOR HEALTHCARE DEVELOPMENT & YOUTH EMPOWERMENT

VISIBILITY, INCLUSION AND RECOGNITION OF INTERSEX PERSONS IN CAMEROON

A Baseline Study on The Public Perception of Intersex Persons and Intersex Genital Mutilation in Cameroon



By Intersex Nigeria
(Centre for Healthcare Development and Youth Empowerment)

Contact Information

Intersex Nigeria (Center for Healthcare Development and Youth Empowerment)

Lagos and Abuja, Nigeria

info@intersexnigeria.org

+2349129057794

Visit our website

www.intersexnigeria.org

Social Media

@Intersex_nigeria_

@IntersexNigeria

@IntersexNigeria

Intersex Nigeria

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FOREWORD

We would like to make it clear from the outset that we were aware of our very partial knowledge of the intersex issue at the time this research was set up. Although we were activists in the field of sexual and reproductive rights as queer feminists, the intersex issue seemed distant and far removed from our own realities. Nor was it possible to include in the team an intersex person whose experience, vision and approach would have enabled us to better structure the survey.

In the course of this survey, we were able to deconstruct a large number of preconceived ideas, and we're still in a learning phase. It is therefore possible that the way in which certain terms or expressions are used may not meet with consensus among associations campaigning for the rights of intersex people. We apologize in advance, and ask you to read this report with the humility befitting non-specialists.

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INTRODUCTION

As part of the implementation of the VIRAL project (a project initiated by Intersex Nigeria) to combat genital mutilation of Intersex people in Cameroon and Nigeria, exploratory research was carried out on the situation of Intersex people in Cameroon. This report summarizes the findings of the exploratory research, and will focus on three main areas:

- An overview of the legal, medical and sociological situation of intersex people in Cameroon. It is structured in four parts. The first presents the context, objectives and methodology; the second presents the legal aspects; the third presents the medical aspects; and the last presents the sociological aspects.

- Perceptions and myths of intersex people in Cameroon; this involves developing the ideas that people have of intersex people and how these impressions impact the lives of intersex people themselves.

- The prevalence of genital mutilation among intersex people in Cameroon. A survey that requires not only appropriate logistics, but also a detailed knowledge of epidemiology, which none of the researchers on this project possess. Studying the prevalence of a phenomenon means measuring its frequency at a given time, in a given geographical area and in a given population. This presupposes a minimum of information for each individual over a given period.

In the absence of precise information on birth registers in Cameroon (the World Bank estimates that only 40% of births are registered), and in the absence of documentation on intersex people in Cameroon, we can only rely on the combined experiences of health professionals and intersex people themselves. By genital mutilation we mean any intervention aimed at modifying the genital organs, whether or not it takes place in a medical setting. In fact, this report gives only a very vague overview of genital mutilation on intersex people. This is partly because the operations performed on babies born with variations in their sex are not perceived as attacks on physical integrity, and partly because we met few people who could testify to these operations.

a- Background to the study



The binarity of sex and gender into men and women is an invariable feature of almost all human societies. Anyone who deviates from this pattern is rejected and ostracized. Those who display atypical sexual characteristics from birth or adolescence are commonly but incorrectly referred to as "hermaphrodites". This term, which is specific to plants and animals, is often used to designate people who exhibit rather atypical or even ambiguous genital development, i.e. genetic, chromosomal, anatomical, gonadal or hormonal variations that do not coincide with the classic categorization of male or female, masculine or feminine. According to experts, just under 2% of the world's population is born with intersex characteristics. In countries where medical interventions are highly advanced and care accessible, it is very common for children born with these characteristics to undergo surgery or other interventions and treatments with the aim of "harmonizing" their sex and appearance to conform to binarity norms.

In Cameroon, there is very little documentation on the intersex issue. Apart from a few cases reported by the media in the form of news items dominated by embarrassment, violence and often a great deal of pain , it is difficult to find information on intersex people. For example, it's impossible to know how many children are born with this particularity, or what their experiences are, from infancy to adulthood. There are many myths and misconceptions about who intersex people are, making the subject taboo and research difficult. It is to fill this void that this research is being carried out.

b- Study objectives



Get a clear picture of the legal, medical and social situation of intersex people in Cameroon.
Gathering and analyzing the different perceptions of intersex people in Cameroonian society.
Draw up an inventory of existing mechanisms and tools for supporting intersex people.Get a clear picture of the players, structures and partners working with intersex people, as well as the prevalence of genital mutilation and its risks and consequences.

2. Methodology



To achieve these objectives, we opted for a mixed methodology combining qualitative and quantitative approaches.

The quantitative approach consisted in measuring the state of knowledge about intersex people using questionnaires. Two separate questionnaires were administered, one aimedspecifically at healthcare professionals, the other at the general population. The criteria for participation were relatively open-ended, enabling nearly 1,000 respondents to take part, spread across all the major towns in Cameroon. All questionnaires were administered faceto-face. Questionnaire data were processed in Excel. These data will be the subject of a separate report on the myths and perceptions of intersex people in Cameroon.

The qualitative approach consisted of: a documentary review of the various texts and publications directly or indirectly concerning intersex people in Cameroon; individual interviews with a wide range of profiles, including intersex people, their relatives or parents, doctors, journalists, administrative and legal authorities such as jurists, lawyers and police officers; and finally, two collective meetings (in progress), the first of which will be an open discussion between these different players on the situation of intersex people in Cameroon.

The second meeting will be a presentation of the results and validation of the report in thepresence of the people who have actively participated in this research.

The qualitative data was processed in the classic way, taking care to note highlights, life stories and points of recurrence and cross-referencing between different data. In this way, we make extensive use of interview-inspired verbatim to illustrate our analyses.

More specifically, to obtain results on the prevalence of genital mutilation, our work is based on a composite material. Data were extracted from a quantitative survey on the state of knowledge and perception of intersex people; we specifically extracted data from health personnel and traditional practitioners. In addition, other more qualitative data were collected from interviews with these same professionals (hospital and traditional medicine); as well as from interviews with intersex people themselves.

2.1 Limits

We have identified several limitations.

From a methodological point of view, the sample of people surveyed is not sufficientlyrepresentative of a population of over 25 million. However, given that this was an exploratory survey carried out in the country's ten regions, and given that the issue is still little known to the general public, the data collected can be used as an advocacy tool. What's more,as the intersex issue is new, at least in these specific terms, the fact that the interviewers had to explain from the outset what it was all about biased the analysis of the state of knowledge.

For example, before administering the questionnaire, the interviewer had to explain what it was all about. In fact, when asked whether or not the respondent had ever heard of the term ntersex, the answer was yes.

It was not easy to mobilize intersex people themselves in this survey; this may be due to a lack of communication, but it is also possible that the short time devoted to the survey and the means made available were not sufficient.

Another limitation is linked to the team's capabilities, particularly in terms of statistics. For example, it has not been possible to carry out a specific study to determine the prevalence of genital mutilation among intersex people, as the project would have wished. It is, however, possible to assess the extent of surgery performed on intersex people by health professionals.

3. ANALYSIS OUTCOME

3.1 CURRENT SITUATION OF INTERSEX PEOPLE IN CAMEROON



3.1.1 Between legal vacuum and abuse. The absence of a legal framework

Cameroonian law is silent on the phenomenon of intersex. When a child is born with sexual variations that are difficult to classify as either male or female, parents and registrars generally choose to register one sex or the other as a matter of form, even though there is nothing to suggest that the child would develop the characteristics associated with either the female or male gender. In some rare cases, for example where it is common practice not to register children at birth but at a later date, the birth certificate is simply not issued. This means there are intersex children without a birth certificate, even though it is an important piece of documentation, a fundamental right from which many other rights derive. Without a birth certificate, it is not possible to be recognized by the state. It is not possible to access basic services such as education, health and justice.

This is the situation experienced by Claude, née Rosalie. The law makes no provision for the recognition of a gender other than male or female. He is over thirty, has no identity papers and has never been able to obtain any official documents because he has no birth certificate. Claude has both characteristics, and despite numerous explanations of the conditions of his birth, this fundamental right has not been recognized.

For intersex people whose birth certificate mentions a gender that is ultimately contradicted by the child's development during adolescence and/or adulthood, only a judge's decision can rectify the gender mentions. Name rectification is decided by the President of the Republic.

The establishment of birth, marriage and death certificates is governed by ORDINANCE No. 81/002 of JUNE 29, 1951, on the organization of civil status and various provisions relating to the status of natural persons. A judge's decision is required to rectify an entry on a birth certificate, or to reconstitute a birth certificate if it was not drawn up within the first 60 days of a child's life. In Cameroon, birth certificates are reconstituted in the event of loss or destruction of registers, or when the declaration has not been made to the civil registrar within sixty days of the birth by the head of the hospital or the doctor when the child is born in hospital, or by any other person who assisted the mother of the child, or later by the child's parents.

When a birth has not been declared within six months, it can only be registered by the civil registrar by virtue of a judgment handed down by the competent court under certain conditions.

This is a judgment which allows the birth certificate to be reconstituted or rectified. This procedure is rarely used, given the cumbersome conditions, slow procedures and cost.
As a result, intersex people use birth certificates that are either falsified, false or illegal. While this temporary solution allows access to certain services, it remains precarious and unsustainable in the long term. For example, it is not possible to take part in official competitions, obtain a passport or open a bank account. These are all fundamental rights to which intersex people have no access because their existence is not legally recognized.

For those who do have a birth certificate and identity document, it's often when it comes to renewing these documents that problems arise. Some agents require a court ruling when appearance does not correspond to the sex indicated on the birth certificate, and very often subject applicants for this public service to humiliating and degrading treatment, such as demanding to see private parts, or publicly asking indiscreet questions.

Government officials, like the rest of society, are uninformed and poorly trained about intersex people. Claude has been arrested by the police on several occasions and taken to court by a prosecutor who accused him of identity theft.

3.1.2 A poorly informed and poorly trained medical community

The results of our exploratory survey of healthcare and related professionals - doctors (gynecologists, surgeons, pediatricians, psychologists), public health experts, nurses, midwives, sexologists, pharmacists, medical students, etc. - show that out of 188 people questioned, 134 have heard of the term "intersex", giving a 71% awareness rate of the phenomenon. On the other hand, despite having heard of the term intersex, many continue

to use the term "hermaphrodite", and 23% have an erroneous knowledge of what an intersex person is. They confuse it first with transidentity, and then with homosexuality. 14% of these professionals think it's absolutely necessary to operate from birth on the basis of the dominant sex, while around 30% think it's best to let them choose when they're teenagers or adults before operating. However, for more than 50%, intersex people should be left as they are, given the consequences that operations can have, and instead of operating, psychological support should be offered. Two health professionals, however, felt that intersex should be banned.

Cameroon is no exception to the pathologization of intersex identity. In the survey of health professionals, more than half consider it to be a malformation (to be corrected). In the majority of cases, operations are carried out on newborns and sometimes on teenagers, without urgency or serious medical necessity (these children are in good health and their organs function perfectly), with not only risks of complication (no medical operation is non-invasive and benign) but also sometimes dramatic physiological and psychological consequences. When affected organs malfunction, for example, or when, as adults, these children fail to develop the desired characteristics. Clarisse (nickname) is now 28, has never menstruated and her breasts have never developed. In her case, her parents chose to have her penis removed. Some health professionals take these operations "for granted", believing that intersex people must be assigned a legal and social gender binarity at all costs. Claude explains that he himself underwent the operation when he was 3 months old. He still remembers it, as the scars were painful for a long time. Due to a lack of funds, his parents didn't go ahead with the other planned operations. We were able to obtain tele-testimonies of 4 cases of genital mutilation of Intersex people, but we were unable to access the strains. These were mainly minors with visible genital ambiguities. Two of the families decided to operate on their children to make them "girls" and the other two "boys". Unfortunately, in adolescence, the "boys" began to develop breast growths and the "girls" never had breasts, let alone menstruation.

However, it's the parents who sometimes insist on an operation under the pressure of social norms. "I've already seen a case like this. The family wanted to destroy a sex. We advised them, but they insisted and we did the operation," says one nurse.
It turns out that healthcare professionals often lack the training and knowledge to take into account the health needs of intersex people.

In Cameroonian society, health care is provided by two main categories of people: the medical and related personnel mentioned above, and traditional practitioners.

A large proportion of the population entrusts their care to this second category, and the research showed that out of 26 tradipraticians who had ever been in contact with intersex people, 21 said they had heard of intersex people, i.e. 77.7%. However, none of them were able to define the term correctly: 8 said it referred to people with 2 sexes, while 5 thought it referred to people with abnormalities in their sex, or who developed secondary sexual characteristics contrary to their sex - "for example, a woman with a beard". One person stated that there was no such thing as an intersex person, while 4 tradipraticians were convinced that these were people who were attracted to both men and women.

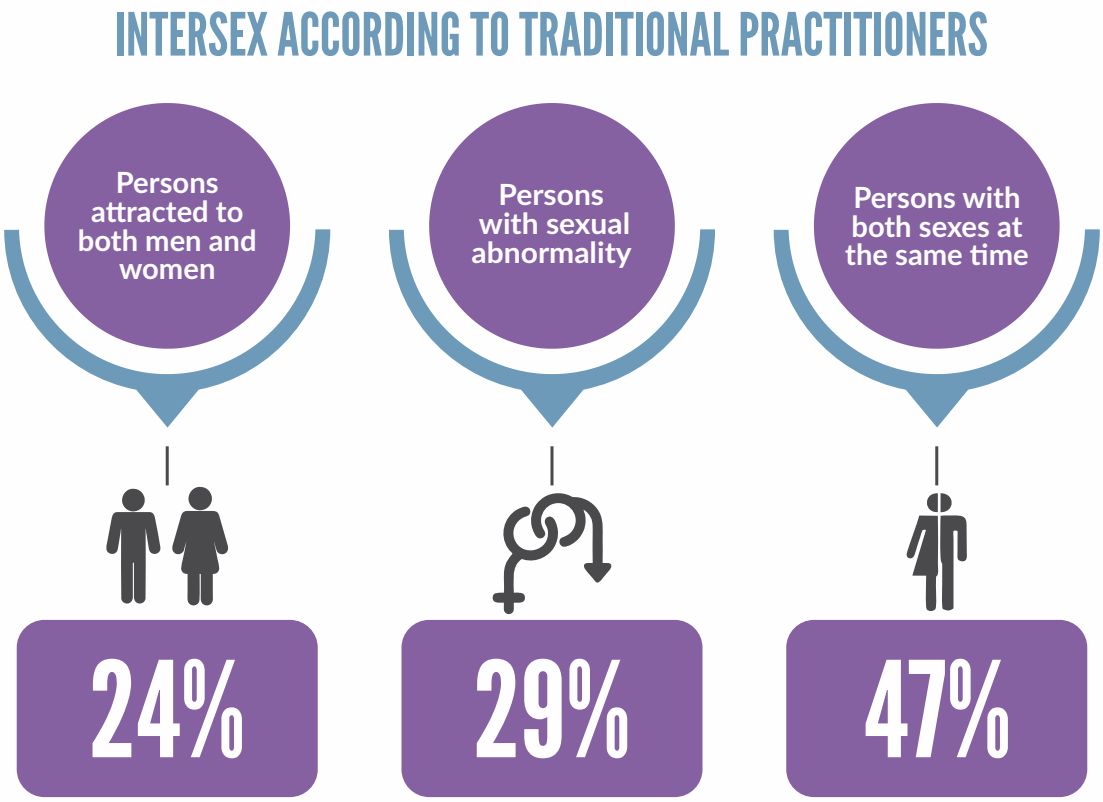


Figure 1

75% of people who have never heard the expression intersex think that it's people attracted to both men and women, while one person thinks that "it's a man with a vagina", so 75% of these people have heard the expression hermaphrodite.

50% of the tradipraticians we met said they had already consulted intersex people. Opinions were mixed, the vast majority having been contacted by parents who wanted their child to be freed from what they believed to be a curse. While some indeed advised "killing one of the sexes", most said it was dangerous to do so, and advised families to let the children grow up and make their own choices. One person, however, thought it best to "exterminate them".

3.1.3 Silence, shame, rejection, stigmatization, discrimination: a shared social experience

What characterizes intersex people in Cameroon socially is above all a shared experience of silence, shame, rejection, stigmatization and discrimination.

When someone is born, the first question is often what sex the child is. The difficulty of distinguishing the child's sex leads to the first silences on the part of parents. Babies are the object of curiosity from the very first months of life. Claude explains that, in his case, his mother avoided entrusting him to other women for bathing, contrary to traditional practice. But the word quickly spread. Playing with other children became difficult because of the mistrust created by parents: "They called me boy-girl and many said I wasn't normal and shouldn't be played with. So I kept to myself. Parents often don't have the resources to explain their child's identity. And if they don't go to doctors, traditional healers, priests or pastors, they choose to hide it to ward off the shame.

Silence is also institutional, starting with the difficulty of establishing a birth certificate. It can also be found in the difficulty of identifying a child and giving it a masculine or feminine name. It continues throughout life, in hospitals in particular, but also at school, in the workplace, etc. *"I realized I was different when I was about 6 or 7, when the other kids laughed at me when we were washing up. My parents dressed me like a girl, but I had a bulge in my underpants like the boys (...) At around 8 I started dressing like a boy. It was better that way (...) It was around the age of 25 that I took matters into my own hands and went to see a gynecologist myself. I needed to understand. That's when they finally took the time to explain to me that I was a woman without a uterus. I look like a man, but my breasts have grown, so my nephews call me Auntie" (LC, 32).*

When the general population was asked whether they thought intersex people were normal, 52% said they did, 24% said they didn't, and around 3% had no idea. 335 people know at least one intersex person, i.e. around 35%. This proportion includes the highest number of people who think they are normal (208, or over 62%). However, even among those who know or have met an intersex person, 120 feel that they are not normal, i.e. around 36%.

Table 1. Crossed knowledge of intersex people and perception

	YES	NO
Do you know any intersex person?	335	577
Intersex persons are:	**YES**	**NO**
Normal	208	269
Not normal	120	292
Do not know	7	16

The data from the interview survey contrasts with the life stories of intersex people we collected. They suggest that the majority of the population is aware of what an intersex person is, and of the fact that intersex people should be treated like any other citizen, with respect for their rights. Yet for all the testimonials received, it's a real life-long journey of rejection and discrimination: *"There's never a shortage of spikes. Family, passers-by. Everyone makes remarks"* (JM, 36); "I'm always insulted, even today it's not easy. The little kids make fun of me when I walk by. They say look at the father there with the beard and breasts like a woman"; LC. 37; *"To be called a curse or witchcraft or even a waste of society when you didn't choose to be born that way is really heavy. People say we're destroying society. That we're an abomination. It's hard"* (JM).

All the intersex people we spoke to recount traumatic experiences at school, but above all with the police. They are controlled and humiliated when their appearance doesn't match the gender on their identity papers, if they have any, which opens the door to all kinds of abuse. "What bothers me most is the police. They bully us instead of understanding," explains JM, "all because they want money.

None of the intersex people interviewed has a university degree, nor a high school diploma. While some have had to stop school to protect themselves from ridicule, others have made other choices, motivated either by their passion, as in the case of JM, who would have liked to become a professional footballer, but whose career didn't take off following the many humiliations during the tests.

On the religious front, of the 30 men and women working in the Abrahamic religions (pastor, priest, imam), 27 have already heard of people born with the characteristics of both sexes, although they don't know how to define an intersex person. Of these, 11 people (36%) think that intersex people are not normal. This is a significant proportion within the religious community, and shows just how little is known about this issue.

Table 2: Summary of information on the experiences of intersex people

	Birth Certificate	ID	Academy	Professional Situation	Family Situation	Police Arrest	Surgery
JM	Yes	Yes	Primary	Informal Sector	Family Lives	Yes	No
Claude	No	No	Primary	Informal Sector	Father of Two Children	Yes	Yes (At 3 Months)
LC	Yes	Yes	Primary	Informal Sector	Live Alone	Yes	No

Once again, we'd like to emphasize the great difficulty of meeting Intersex people, the vast majority of whom live a hidden life. Those we've been able to meet and listen to are victims/survivors of violence whose sexual identity has been made public independently of their will, and has been marked by violence and rejection, and today they feel they have nothing left to lose.

Why all this secrecy about a feature that is ultimately intrinsic to the nature of a minority of human beings?

What are the myths and perceptions surrounding intersex people in Cameroon?

3.2 PERCEPTIONS AND MYTHS OF INTERSEX PEOPLE IN CAMEROON



3.2.1 Respondent typology

This exploratory research shed light on people's ideas about intersex people. The exploratory research involved 1,013 people divided into several categories across Cameroon, by gender, age group and socio-professional category.

This distribution covers 9 of Cameroon's 10 regions:
The people we interviewed formed the basic sample for this exploratory research and belong to several socio-professional categories that we have grouped into the following broad categories:

- Healthcare professionals (doctors and specialists)
- Non-managerial healthcare professionals (nurses, nutritionists, psychologists, etc.)
- Healers, traditional healers and naturopaths
- Religious
- Students
- Teachers
- Legal experts (lawyer and paralegal)
- Unemployed
- Informal sector
- Armed body
- Media
- Social workers

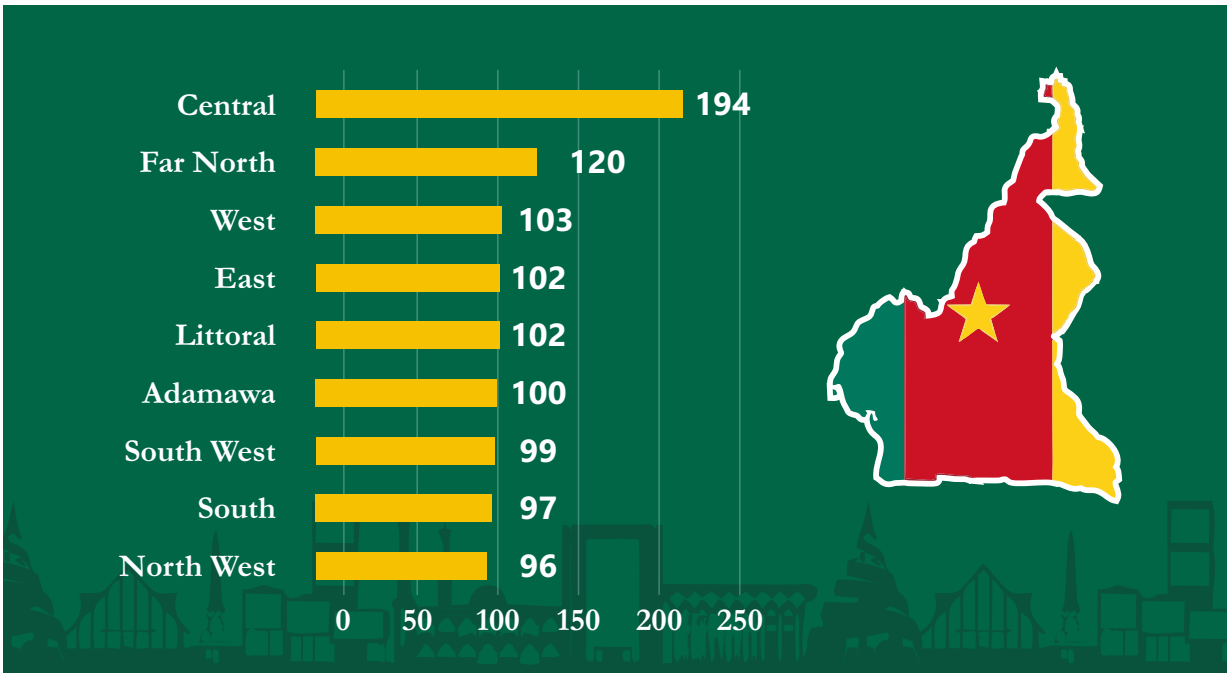


Figure 2

In addition to these socio-professional categories, the respondents range in age from 15 to 70. We've divided the ages into the following groups: 15-20; 21-30; 31-40; 41-50; 51-60; 61-70.

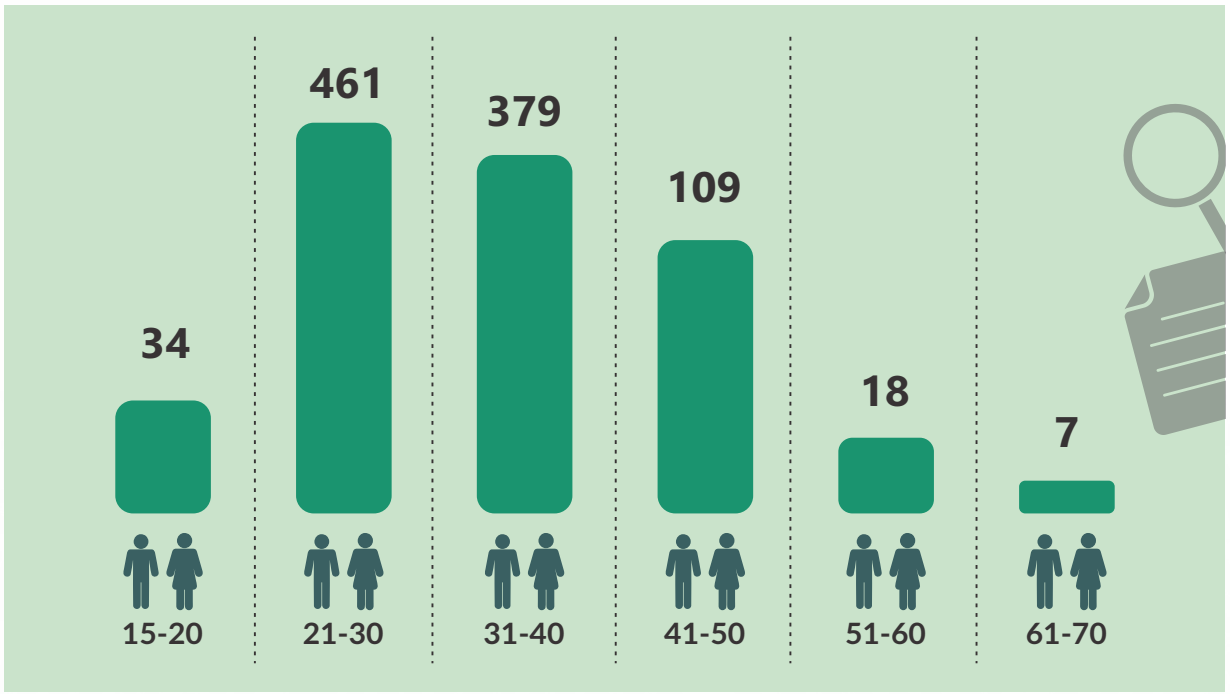


Figure 3

We have divided the ages into these age groups as seen in Fig 4

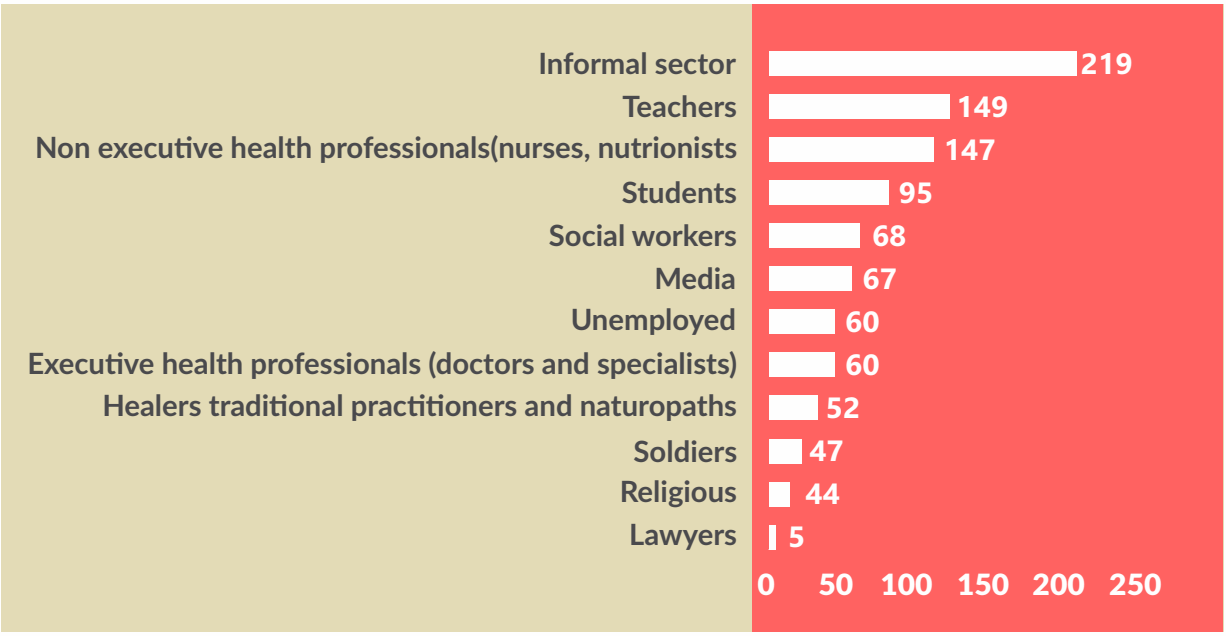


Figure 4

We found that the age groups that participated most in the research were 21-40 year olds.

I Is the term intersex familiar to Cameroonian vocabulary?

To answer this question, we asked the sample a series of 03 questions.

II Have you ever heard of intersex people?

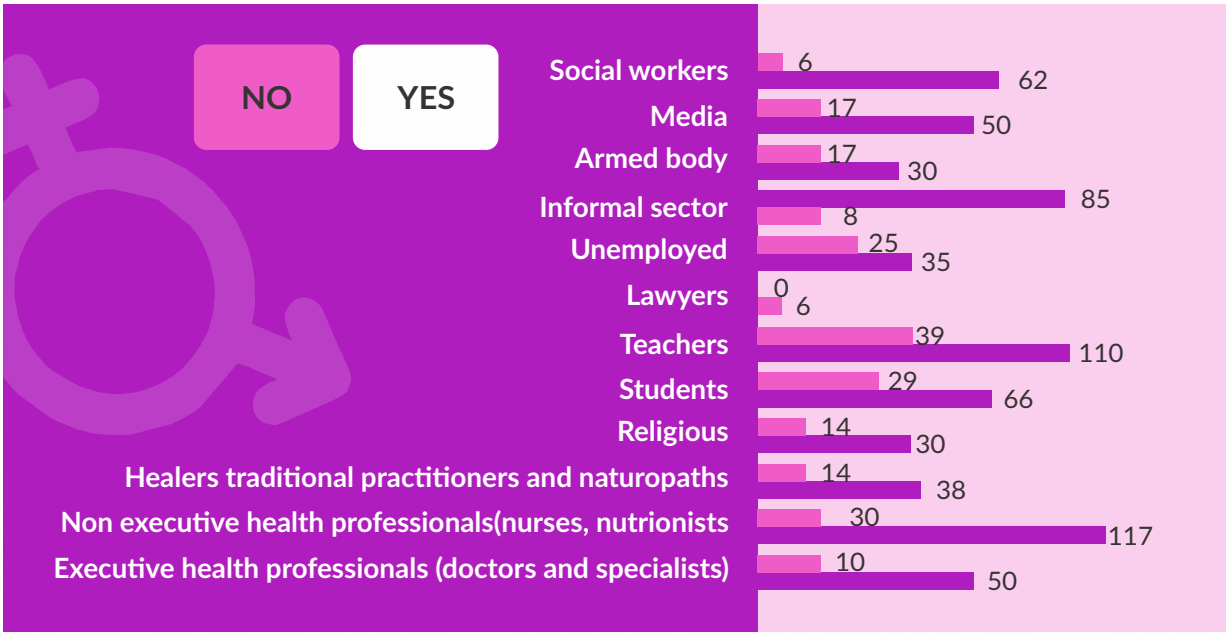


Figure 5

Overall, 39.33% of those questioned had never heard of the term intersex. Do the 60.7% who have heard of it really know what it means?

III Have you ever heard of so-called hermaphrodites?

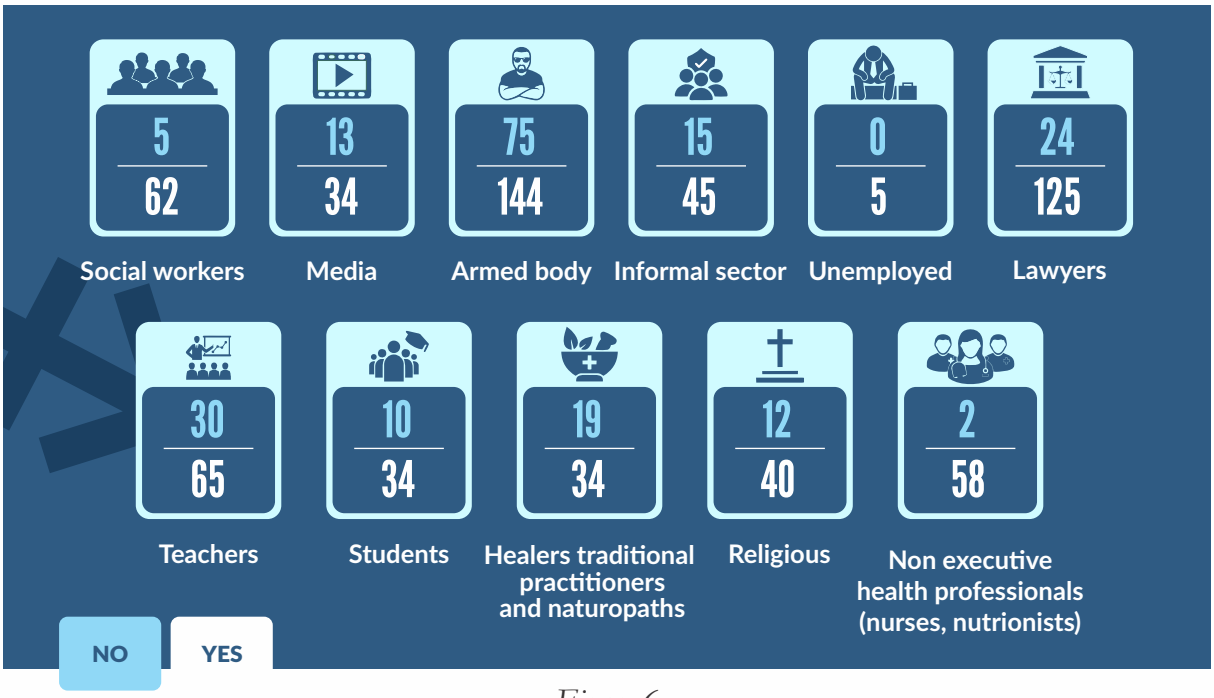


Figure 6

75% of those questioned had heard of so-called hermaphrodites, i.e. 14.3% more than those claiming to be familiar with the term intersex.

IV Have you ever heard of people having two sexes?

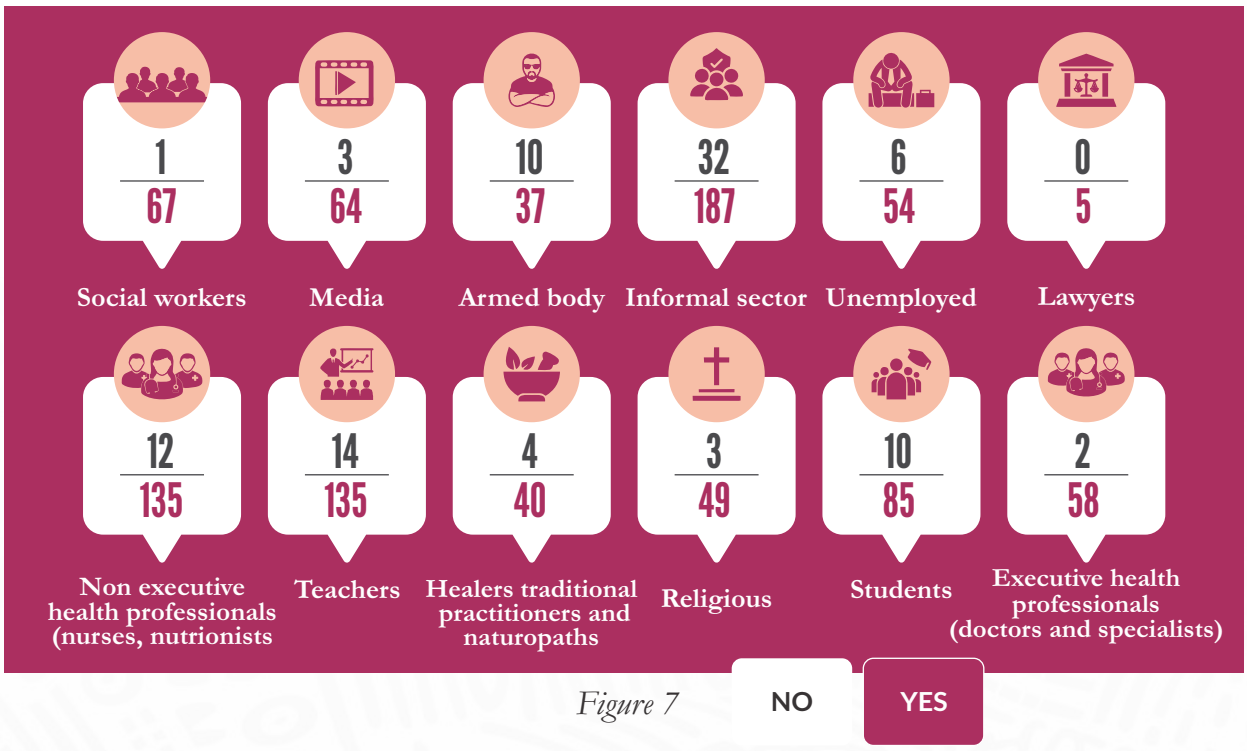


Figure 7

Given the results of the research, 90% of Cameroonians surveyed are closer to the expression "people with two sexes", proportionally fewer people are familiar with the expression "hermaphrodites", and 30% fewer with the expression "intersex".

This analysis enabled us to understand that in the understanding of the people interviewed, the perception of sexual identities remains very binary. Either you have one sex (male/female), or you have two in the case of intersex. Even so, the notion of intersex remains very mixed within the Cameroonian community, depending on one's socio-professional background, education or where one lives.

3.2.3 Between myths, popular beliefs and reality

I The Cameroonian definition of intersex



In order to obtain answers as close as possible to reality, open-ended questions were asked without any suggested answers.

So we have several definitions of the term intersex, ranging from people with two sexes, to witches, to homosexuals.

- The right answers. These were the answers that were in line with the UN definition of intersex as "a way of describing an individual's biological sex characteristics, including genitalia, gonads, hormone levels and chromosomes" when these characteristics do not correspond to traditional definitions of male or female sex. An example of a correct answer would be: A person who has both sexual characteristics, whose characteristics do not correspond to the norms; who presents both feminine and masculine characteristics, and so on.

- Partial answers, Example: a person who is neither male nor female; congenitally malformed, etc.
- False Answers, Example: a homosexual, no sexual orientation, witchcraft etc.

- Null answers, those who don't know.

Table 3

	Just	Partial	Fake	Null
Healthcare professionals (doctors and specialists)	18	30	7	5
Non-managerial healthcare professionals (nurses, nutritionists, psychologists, etc.)	36	63	31	17
Healers, traditional practitioners and naturopaths	9	22	12	9
Religious	5	17	11	11
Students	25	27	20	23
Teachers	16	60	34	39
Legal experts (lawyers and paralegals)	1	3	1	0
Unemployed	4	23	19	14
Informal sector	32	78	33	76
Armed body	7	16	11	13
Media	16	27	14	10
Social workers	25	31	7	5

Only 23% of people who claim to know and have been in contact with intersex people have the correct definition of intersex or intersexuality.

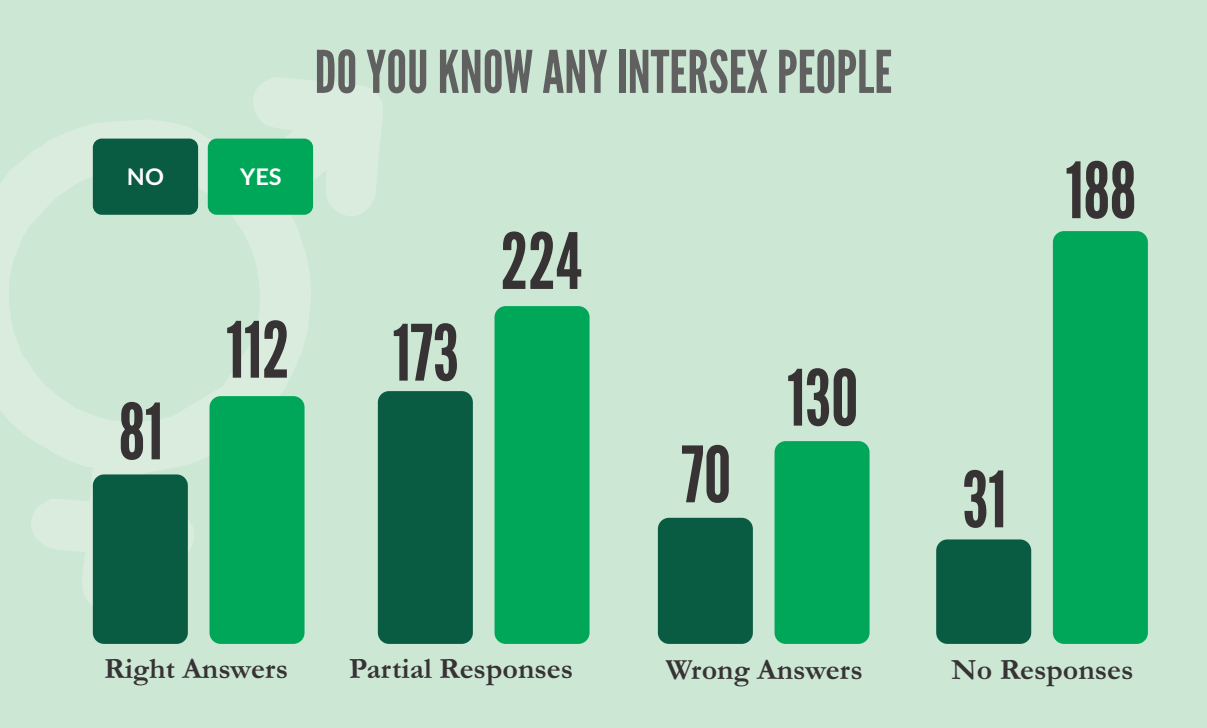


Figure 8

If we focus on people who know intersex people, and we have more than 75% partial or false answers, we understand the mystery that hangs over this community.

According to interviews with intersex people, 65% define themselves as having two genders, while 35% define themselves as Trans. Those who define themselves as having 2 genders have tried to undergo surgery to retain just one gender, with the help of doctors.

3.2.4 Intersex perceptions

The exploratory research enabled us to classify respondents' perceptions into three main categories:

- Negative perceptions
- Positive perceptions
- Neutral perceptions

Negative perceptions here refer to the demeaning, degrading ideas that people have about intersex people. These include, but are not limited to, ideas that intersex people are cursed, have mystical powers, are homosexual , or have unbridled sexuality. On the whole, whether mystical or scientific, the people interviewed here believe that intersex people are sources of imbalance in the natural order.



Figure 9

To understand perceptions, and above all their impact on the lives of intersex people and their loved ones, we asked respondents what should be done for intersex people. Somewhere between a curse and an exorcism, we noted a great deal of empathy, listening and understanding.

Age range	15-20	21-30	31-40	41-50	51-60	61-70
Curse, discrimination, marginalization	4	27	11	4	0	0
Support, empathy, understanding, listening	9	195	180	45	10	0
Medical operation	10	47	37	15	4	1
Exorcism	0	9	7	2	1	0
Let live with respect	3	41	22	18	0	0
Information, company training	2	28	3	0	1	0
RAS	6	111	102	22	3	2

We note 43% empathy, understanding and listening, which is a good basis for initiating awareness-raising, advocacy and training, in order to not only improve understanding of intersex, but also deconstruct preconceived ideas, which have a considerable negative impact on intersex people.

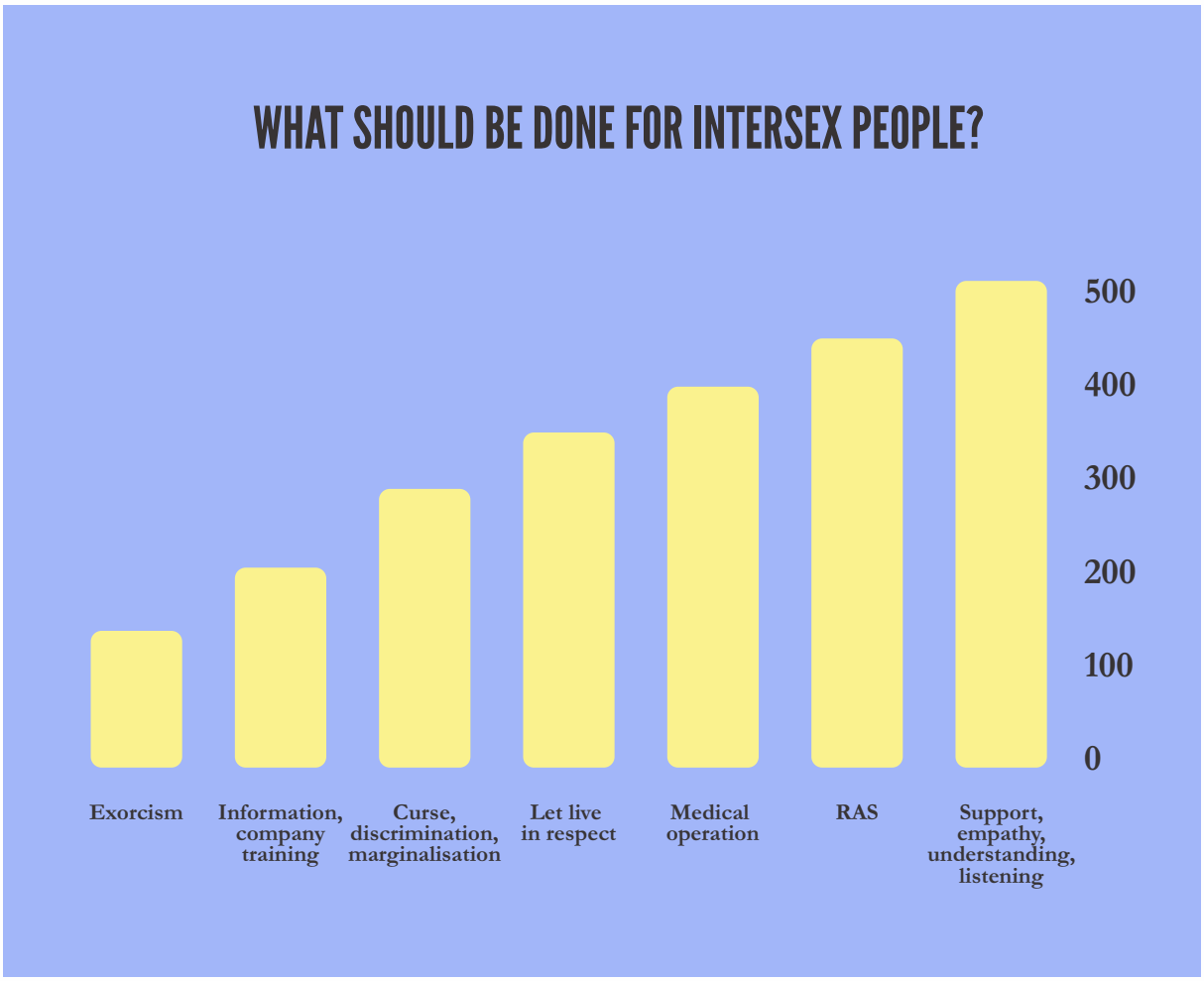


Figure 10

Even if exorcisms, curses and medical operations still account for a fairly large proportion of the opinions collected (18%), having the information linked to the age groups and socio-professional categories concerned, it will be easier to develop appropriate communication and awareness-raising tools.

Beyond myths and perceptions, it's important to understand the phenomenon of genital mutilation of intersex people, its recurrence, effects and impact.

3.3 PREVALENCE OF GENITAL MUTILATION AMONG INTERSEX PEOPLE IN CAMEROON

3.3.1 A trivialized surgical act VS non-integrated violence



The results of our exploratory survey of healthcare and related professionals, including doctors (gynecologists, surgeons, pediatricians, psychologists), public health experts, nurses, midwives, sex therapists, pharmacists, medical students, etc. show that 14% of these professionals think it's absolutely necessary to operate on the basis of dominant sex from birth, while around 30% think it's best to let them choose when they're teenagers or adults before operating. This makes a total of 44% of hospital health professionals who believe that surgery is necessary. However, the study also showed that around 50% of professionals believe that intersex people should be left as they are, given the consequences that operations can have, and that instead of operating, psychological support should be offered. Two health professionals, however, felt they should be banned.

Of these 188 healthcare professionals, 60 are doctors and specialists. Of these 60 professionals, 40 believe that intersex is natural and somehow normal. 31 believe that intersex people need to be listened to and empathized with, while 4 think that gender reassignment surgery is necessary at all costs. Some 37% of doctors have already seen intersex people in consultation, 9 of them more than once, specifically in the cities of Yaoundé and Douala. 75% of these professionals believe that intersex is a peculiarity to be accepted.

Few doctors report having operated on intersex children. Of the 25 or so intersex patients he has seen over the course of his career, one surgeon claims to have operated on fewer than ten, often at the insistence of the parents, often because of a malfunction of the organs, such as for peeing. All the operations performed were on children, newborns and children under 5 years of age.

It seems clear that in the medical sector, operations do not appear to be systematic, even

though some professionals do not necessarily see them as a form of violence, but rather as a surgical act of repair. Nevertheless, this raises a number of questions that need to be addressed. What does the fact that few doctors, including pediatricians, have consulted intersex people mean? Given the proportion of professionals who have no problem with performing operations, why are so few actually talking about it? Our hypothesis is that further research is needed to determine whether this is a matter of omerta between professionals, a problem of access to health services, or voluntary avoidance of health services on the part of the people concerned and/or their parents.

As far as traditional practitioners are concerned, no experience of mutilation has been reported. Interviews reveal that when they are confronted with such cases, which are very rare and always the work of parents, they prefer to refer the children to hospital if the parents want surgery. Otherwise, they simply suggest other treatments to improve the mental and spiritual health of their patients. It is difficult to confirm this information at this stage of the survey.

In fact, 02 of the 52 traditional practitioners interviewed thought that intersex people should be killed at birth, 4 advocated spiritual treatments to exorcise the curse - most of the time at the request of the parent - and 50% of the latter thought that these traditional treatments should be able to "kill" one of the sexes and enable the child to be more fulfilled. While only 05 traditional practitioners clearly believe that it is important to carry out mutilation with the help of the medical profession, the interviews gave the impression of something being left unsaid. The tradipraticians interviewed emphasized that they were not qualified to perform this type of operation, and that only doctors could do it. None of the traditional practitioners interviewed admitted having performed an operation on an intersex patient. As a recommendation, it would be important to work specifically with traditional practitioners, who, after the medical profession, have had the most contact with intersex people. 2 out of 3 traditional practitioners have already been consulted on the case of an intersex person.

What about intersex people themselves?

3.3.2 Genital mutilation, a reality for 1 in 3 people

One of the 3 people we interviewed claimed to have undergone mutilation at 3 months. These operations could have continued if his father had had more means. One person out of three is still a significant proportion, and it's quite revealing of the extent of mutilation on intersex people. While the other two intersex people were not operated on, and one doctor deplored the fact that one of them could not be operated on at birth, mutilation really is a reality in the lives of intersex children and adults in Cameroon.

This study would benefit from further reflection with intersex people themselves. One of the major difficulties of this research was to gather testimonies from intersex people.





3.3.3 Genital mutilation among intersex people, its risks and consequences

During the exploratory research, a large proportion of the population defined an intersex person as someone with two sexes. As such, among other proposals made, people mentioned the removal of one of the two sexes, either surgically or mystically, and this is not without consequences, according to the doctors, traditional practitioners and intersex people interviewed.



I) Medical consequences of genital mutilation for intersex people

Intersex people undergo sex-assignment operations that are experienced as genital mutilation and an attack on their physical and psychological integrity.

This mutilation takes the form of surgical interventions and the administration of drugs.

These mutilations are not without consequences and among the medical consequences, we can mention :

- **Immediate consequences:**
- The pain
- Surgical wound infection,
- A hemorrhage

Late consequences are :

- Modification of the external genitalia and reproductive system
- In some cases, lifelong hormone replacement therapy is instituted.

  This treatment can also increase the risk of thrombo-embolic disease,
- Insensitivity to hormonal treatment, particularly to androgens, due to a defect in the function of the androgen receptor.
- Congenital adrenal hyperplasia due to limited production of hormones in the adrenal glands. This affects the child's growth and development.

  Untreated or inadequately treated, it can lead to death.
- From a sexual point of view, they can result in pain during intercourse due to poor healing of the surgery on the sexual organs, or erotic insensitivity.
- The surgical procedure may also result in a vesico-urethral fistula, leading to urinary incontinence.



II Socio-psychological consequences

We were able to report the following risks:

- Complete or partial paralysis of the person
- Sterility
- The death
- Removal of the "bad" sex
- Sexual problems (impotence/frigidity)



The most recurrent consequences were:

- Psychological problems
- The physical discomfort of being in the wrong body
- Social maladjustment
- The strong need to hide oneself and one's body, linked to incomprehension and discomfort.



CONCLUSION

Our survey shows that few professionals claim to have performed sex reassignment procedures on intersex people, whether they are specialists in hospital medicine or traditional medicine. However, one in three intersex people claim to have undergone an operation at birth. Mutilations and their consequences are certainly underestimated, due to the lack of visibility of intersex people.

The original aim of this survey in Cameroon was only to measure the prevalence of genital mutilation among intersex people. For this reason, most of the data is based more on an overview of the situation of intersex people and perceptions, than on epidemiological data. It is essential to carry out a prevalence study under conditions appropriate to epidemiological studies.

Moreover, it is not possible to know how many children are born intersex in Cameroon. Firstly, because there are still problems with birth registration. According to the World Bank, the birth registration rate for children under 5 is around 40%. Secondly, the obligation to state an F or M sex pushes parents to choose a sex for their children born with sex variations that are difficult to classify. Sometimes parents have to wait and see which sex characteristics will dominate. But this is not always obvious. Then, after the legal 60-day registration period for all births, a judge's decision is needed to establish a birth certificate. A long and sometimes costly procedure. In fact, $^{1/3}$ of the intersex people interviewed have no identity papers, and none of them have an identity document that conforms to their identity.

Most have barely completed primary school and live in informal occupations, adding to their precariousness. Although the majority of doctors believe that it is right to wait for people to choose for themselves the solution best suited to their needs, a significant proportion still prefer to resort to or recommend mutilation. The degree of prejudice and preconceived ideas about intersex people is particularly high, demonstrating the extent of the information, training and communication work needed on the intersex phenomenon in Cameroon.

RECOMMENDATION

For Health Professionals and Traditional Practitioners



- Implement mandatory training programs for health professionals (including doctors, surgeons, pediatricians, psychologists, and nurses) on intersex, its variations, and the ethical considerations surrounding surgical interventions.
- Prioritize psychological support for intersex individuals and their families over surgical interventions. Train health professionals to provide empathetic and non-judgmental counselling.
- Ensure that any medical interventions are only performed with the informed consent of the intersex individual when they are of an age to make such decisions. Discourage early surgical interventions unless medically necessary.
- Establish clear guidelines and protocols for the management of intersex variations that prioritize well-being.
- Facilitate collaboration between traditional practitioners and medical professionals to ensure that intersex individuals receive appropriate care and are not subjected to harmful practices.
- Conduct workshops and information sessions for traditional practitioners to educate them about intersex variations and the risks of genital mutilation. Emphasize the importance of referring cases to medical professionals when necessary.

Policymakers and Intersex Advocacy Groups



- Enact laws that protect intersex individuals from non-consensual genital mutilation and ensure their rights to bodily autonomy and integrity.
- Improve birth registration processes to accurately document intersex births and ensure that intersex individuals receive identity documents.
- Launch public awareness campaigns to educate the public about intersex variations, aiming to reduce stigma and misconceptions.
- Establish support networks and resources for intersex individuals and their families to provide guidance, information, and community support.

REFERENCES

1. Ammaturo, Francesca Romana. 2016. Intersexuality and the "Right to Bodily Integrity". *Social & Legal Studies* 25: 591–610.
2. Bauer M, Truffer D, Crocetti D (2020) Intersex human rights. *The International Journal of Human Rights* 24(6): 724–749.
3. Bauer, Markus, Daniela Truffer, and Daniela Crocetti. 2020. Intersex Human Rights. *International Journal of Human Rights* 24: 724–49.
4. Boladale, M., Olakunle, O., Olutayo, A., et al. (2015) Sexual Orientation and Quality of Life among Students of Obafemi Awolowo University (OAU), Nigeria. African Health Sciences, 15, 1065-1073. https://doi.org/10.4314/ahs.v15i4.3
5. Carpenter M (2012) Intersex: Intersectionalities with disabled people. Available at: https://ihra.org.au/21214/intersex-and-disability (accessed 29 June 2021).
6. Carpenter M (2016) The human rights of intersex people: Addressing harmful practices and rhetoric of change. *Reproductive Health Matters* 24(47): 74–84.
7. Carpenter M (2019) Joint Statement on the International Classification of Diseases 11. Available at: https://ihra.org.au/35299/joint-statement-icd-11/ (accessed 29 March 2022).
8. Carpenter M (2020) The OHCR background note on human rights violations against intersex people. *Sexual and Reproductive Health Matters* 28(1): 13–16.
9. Carpenter M (2020a) Intersex human rights, sexual orientation, gender identity, sex characteristics and the yogyakarta principles plus 10. *Culture, Health and Sexuality* 23(4): 516–532.
10. CoE (Council of Europe Commisioner for Human Rights). 2015. Human Rights and Intersex People. Issue Paper. Strasbourg. Available online: https://book.coe.int/en/commissioner-for-human-rights/6683-pdf-human-rights-and-intersex-people.html (accessed on 5 January 2023).
11. Crocetti, Daniela, Elisa A. G. Arfini, Surya Monro, and Tray Yeadon-Lee. 2020. "You're Basically Calling Doctors Torturers": Stakeholder Framing Issues around Naming Intersex Rights Claims as
12. Crocetti D, Monro S, Vecchietti V, et al. (2020) Towards an agency-based model of intersex, variations of sex characteristics (VSC) and DSD/DSD health. *Culture, Health and Sexuality* 23(4): 500–515.
13. Davis G (2015) *Contesting Intersex: The Dubious Diagnosis*. New York: New York University Press.
14. Davis, Georgiann. 2014. The Power in a Name: Diagnostic Terminology and Diverse Experiences. *Psychology and Sexuality* 5: 15–27.
15. Diamond M, Garland J (2014) Evidence regarding cosmetic and medically unnecessary surgery on infants. *Journal of Pediatric Urology* 10(1): 2–6.
16. Dickens B (2018) Management of intersex newborns: Legal and ethical developments. *International Journal of Gynecology and Obstetrics* 143(2): 255–259.
17. Dreger AD (1998) *Hermaphrodites and the Medical Invention of Sex*. Cambridge: Harvard University Press.

18. Dreger AD (1999) *Intersex in the Age of Ethics. Hagerstown*. Maryland: University Publishing Group.
19. Dreger, Alice D., and April M. Herndon. 2009. Progress and Politics in the Intersex Rights Movement Feminist Theory in Action. *GLQ* 15: 199–200.

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20. , 2023, The Growing Visibility of Intersex Demands at the United Nations: A Review of the Treaty Bodies' Concluding Observations, Doctoral School in Humanities, Social Sciences and Law, University of Granada, 18071 Granada, Spain, *Soc. Sci. 12*(2), 73; https://doi.org/10.3390/socsci12020073
21. Essiben, F., Didjo, C., Koh, V.M., et al. (2019) Adolescent Sexual Behavior in an Urban Area of a Resource-Limited African Country, Cameroon. Open Journal of Obstetrics and Gynecology, 9, 923-935. https://doi.org/10.4236/ojog.2019.96090
22. ECHR (European Court of Human Rights). 2022. M v. France. Application no. 42821/18. Decision of the Fifth Section of 26 of April 2022. Strasbourg. https://hudoc.echr.coe.int/eng#{%22itemid%22:[%22001-217430%22]} (accessed on 27 December 2022).
23. FRA (EU Agency for Fundamental Rights). 2015. *The Fundamental Rights Situation of Intersex People*. Vienna: FRA. Available online: https://fra.europa.eu/en/publication/2015/fundamental-rights-situation-intersex-people (accessed on 27 December 2022).
24. FRA (2015) The Fundamental Rights Situation of Intersex People. Report, FRA-European Union Agency for Fundamental Rights, Austria, April.
25. FRA (2020) EU-LGBTI II: A Long Way to go for LGBTI Equality. Report, FRA-European Union Agency for Fundamental Rights. Austria, May.
26. Garland F, Travis M (2018) Legislating intersex equality: Building the resilience of intersex people through law. *Legal Studies* 38: 587–606.
27. Garland F, Travis M (2020) Making the state responsible: Intersex embodiment, medical jurisdiction, and state responsibility. *Journal of Law and Society* 47(2): 298–324.
28. Ghattas D (2013) *Human Rights between the Sexes: A Preliminary Study on the Life Situations of Inter* Individuals*. Berlin: Heinrich-Boll-Stiftung.
29. Ghattas D (2019) Protecting Intersex People in Europe: A Toolkit for Law and Policymakers. Available at: https://oiieurope.org/wp-content/uploads/2019/05/Protecting_intersex_in_Europe_toolkit.pdf (accessed 12 May 2021).
30. Henry, E., Awondo, P., Fugon, L., et al. (2012) Coming Out of the Nkuta: Disclosure of Sexual Orientation Associated with Reduced Risk Behavior among MSM in Cameroon. Archives of Sexual Behavior, 41, 525-527. https://doi.org/10.1007/s10508-012-9916-8
31. IACHR (Inter-American Commission on Human Rights). 2015. *Violence against Lesbian, Gay, Bisexual, Trans, and Intersex Persons in the Americas*. Official Records. Washington, DC: OAS. Available online: https://www.oas.org/en/iachr/reports/pdfs/ViolenceLGBTIPersons.pdf

(accessed on 10 January 2023).
32. Menguele Menyengue, A.M. (2016) Discourse religieux et homosexualité au Cameroun. Journal des Anthropologues, 146-147, 67-86. http://journals.openedition.org/jda/6494
33. Monro S, Crocetti D, Yeadon-Lee T (2019) Intersex/variations of sex characteristics and DSD citizenship in the UK, Italy and Switzerland. *Journal of Citizenship Studies* 23(8): 780–797.
34. Rubin DA (2017) *Intersex Matters: Biomedical Embodiment, Gender Recognition, and Transnational Activism*. Albany, NY: SUNY Press.
35. Rubin, David A. 2012. "An Unnamed Blank That Craved a Name": A Genealogy of Intersex as Gender. *Signs: Journal of Women in Culture and Society* 37: 883–908. [Google Scholar] [CrossRef] [Green Version]
36. Rubin, David A. 2015. Provincializing Intersex: US Intersex Activism, Human Rights, and Transnational Body. *Frontiers: A Journal of Women Studies* 36: 51–83. [Google Scholar] [CrossRef]
37. Suen YT, Chan RCH, Badgett MVL (2021) The experiences of sexual and gender minorities in employment: Evidence from a large-scale survey of lesbian, gay, bisexual, transgender and intersex people in China. *The China Quarterly (London)* 245: 142–164.
38. Viloria H, Nieto M (2020) *The Spectrum of Sex: The Science of Male, Female, and Intersex*. London: Jessica Kingsley Publications.
39. Ravesloot, Saskia. 2021. The Universal Periodic Review beyond the Binary Advancing the Rights of Persons with Variations in Sex Characteristics. Paper presented at 8th International Conference on Gender & Women Studies, Singapore, July 29; Available online: https://asianstudies.info/wp-content/uploads/2021/09/ProceedingsGWS2021.pdf#page=7 (accessed on 29 November 2022).
40. UN (United Nations). 1989. *Convention on the Rights of the Child*; General Assembly. New York: United Nations Treaty Series. Available online: https://www.refworld.org/docid/3ae6b38f0.html (accessed on 5 January 2023).
41. United Nations, Office of the High Commissioner for Human Rights (OHCHR). 2019. *Background Note on Human Rights Violations against Intersex People*. Geneva: OHCHR. Available online: https://www.ohchr.org/en/documents/tools-and-resources/background-note-human-rights-violations-against-intersex-people (accessed on 10 January 2023).
42. United Nations, Office of the United Nations High Commissioner for Human Rights. 2017. *Reporting to the United Nations Human Rights Treaty Bodies Training Guide. Part I*. Geneva: Manual. [Google Scholar]
43. https://www.journalducameroun.com/cameroun-un-bebe-hermaphrodite-met-sa-famille-dans-lembarras/ , https://www.sourds.net/2008/09/06/la-pathetique-histoire-de-kevin-qui-a-deux-sexes/, https://www.youtube.com/watch?v=Q3QeL8QHrpU.

https://www.journalducameroun.com/cameroun-un-bebe-hermaphrodite-met-sa-famille-dans-lembarras/ , https://www.sourds.net/2008/09/06/la-pathetique-histoire-de-kevin-qui-a-deux-sexes/, https://www.youtube.com/watch?v=Q3QeL8QHrpU,